

MATERIALS AND METHODS

The expert committee

The expert committee comprised dermatologists from the European Academy of Dermatology and Venereology (EADV) Mycology Task Force working group on *Malassezia* folliculitis (MF), with specialized expertise in mycology and MF. Conflict of interest forms were collected throughout the development of this position statement.

Aim of the position statement and target audience

The aim of this position statement is to provide evidence-based recommendations for the diagnosis and management of MF. The target audience includes all healthcare professionals involved in the care of patients with MF. The recommendations are presented through diagnostic and therapeutic algorithms.

Literature review

A systematic literature review was conducted to inform the recommendations; detailed results are available in the supplementary information.

Level of recommendation

The EADV Mycology Task Force working group on MF adopted a grading system (Grades A–D) used by the European Society for Clinical Microbiology and Infectious Diseases to assess the strength of recommendations:

- **Grade A:** Strong support for a recommendation.
- **Grade B:** Moderate support for a recommendation.
- **Grade C:** Marginal support for a recommendation.

- **Grade D:** Support for a recommendation against use.

Expert opinion

The expert panel consisted of 19 members of the EADV Mycology Task Force working group on MF. Consensus was achieved through an iterative process involving a written questionnaire followed by two video conferences. After the first video conference—during which questionnaire responses were reviewed and compared with findings from the systematic review—a draft summarizing discussions and proposed consensus points was circulated. The second video conference finalized the consensus on diagnostic and treatment recommendations.

Consensus

Consensus was defined as agreement by at least 80% of the Task Force members.

RESULTS

Diagnosing *Malassezia* folliculitis

Diagnostic methods for MF are categorized as **recommended** or **supplementary**. Recommended methods are used to confirm or exclude clinical suspicion, while supplementary methods serve as adjuncts in cases with inconclusive results.

Clinical presentation

MF typically presents as 2–4 mm pruritic papules and pustules on erythematous skin, predominantly in seborrheic areas such as the upper arms, chest, back, face, neck, and shoulders. Excoriations and post-inflammatory hyperpigmentation may accompany lesions. Cysts have also been reported in

some cases. The absence of comedones helps differentiate MF from acne vulgaris. Abscesses are not considered typical features of MF.

Direct microscopic examination

Direct microscopy is performed using material from pustules, applied to a glass slide and dissolved in 10–20% potassium hydroxide (KOH). Diagnostic findings include unipolar budding yeasts (blastoconidia), conidia, and occasionally hyphae.

Staining techniques reported in the literature include: - Parker blue - Gram staining - Methylene blue - May-Grunwald-Giemsa (MGG) - Chicago sky blue - Calcofluor white/Blancophor (requires fluorescence microscopy)

Gram staining was evaluated in a study of 26 patients with MF, diagnosed based on at least two of the following criteria: (a) typical clinical presentation, (b) histopathological confirmation of *Malassezia* in inflamed hair follicles, or (c) clinical response to antifungal therapy. The method correctly identified MF in 22 of 26 patients, yielding a sensitivity of 84.6%. All six patients without MF were correctly ruled out, resulting in 100% specificity.